HEALTH ASSESSMENT

Objectives

- At the end of the discussion, the will be able to:
  - Define terms related to health assessment
  - Differentiate nursing assessment from medical assessment
  - Describe the steps of the assessment process
  - Discuss the phases of the nursing interview
  - Identify the components of history taking
  - Conduct general survey
  - Document assessment findings properly

HEALTH ASSESSMENT

Is the collection, validation, organizing and recording of patient’s data on his health status

Nursing Assessment

vs.

Medical Assessment

- **The purpose of a nursing health assessment is to collect holistic subjective and objective data to determine a client's overall level of functioning in order to make a professional clinical judgment.**
- **It identifies human responses to health problems as well as the patient's strengths**
- **This focuses on the disease process, how it affects the patient and its treatment**

Nursing Process

Assessment

- Collect data
- Organize data
- Validate data
- Document data

Diagnosis

- Analyze data
- Identify health problems
- Formulate diagnostic statements

Planning

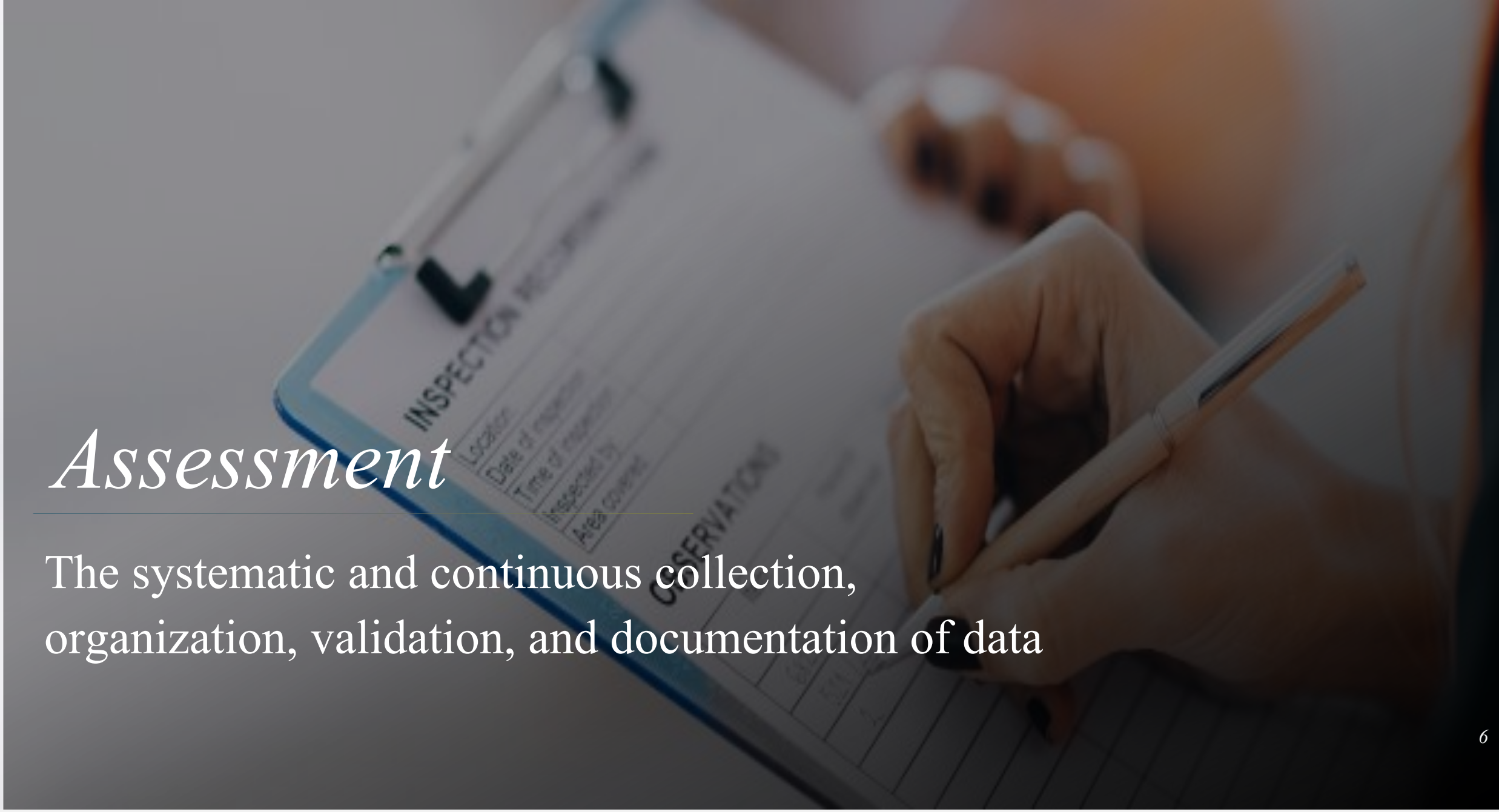
- Prioritize problems
- Formulate goals
- Select nursing interventions
- Write nursing interventions

Implementation

- Render nursing care
- Re-assess client
- Supervise delegated care
- Document nursing activities

Evaluation

- Reassess client
- Evaluate data with outcomes
- Draw conclusions about problem status
- Continue, modify, or terminate the care plan



Assessment

The systematic and continuous collection, organization, validation, and documentation of data

Types of Assessment

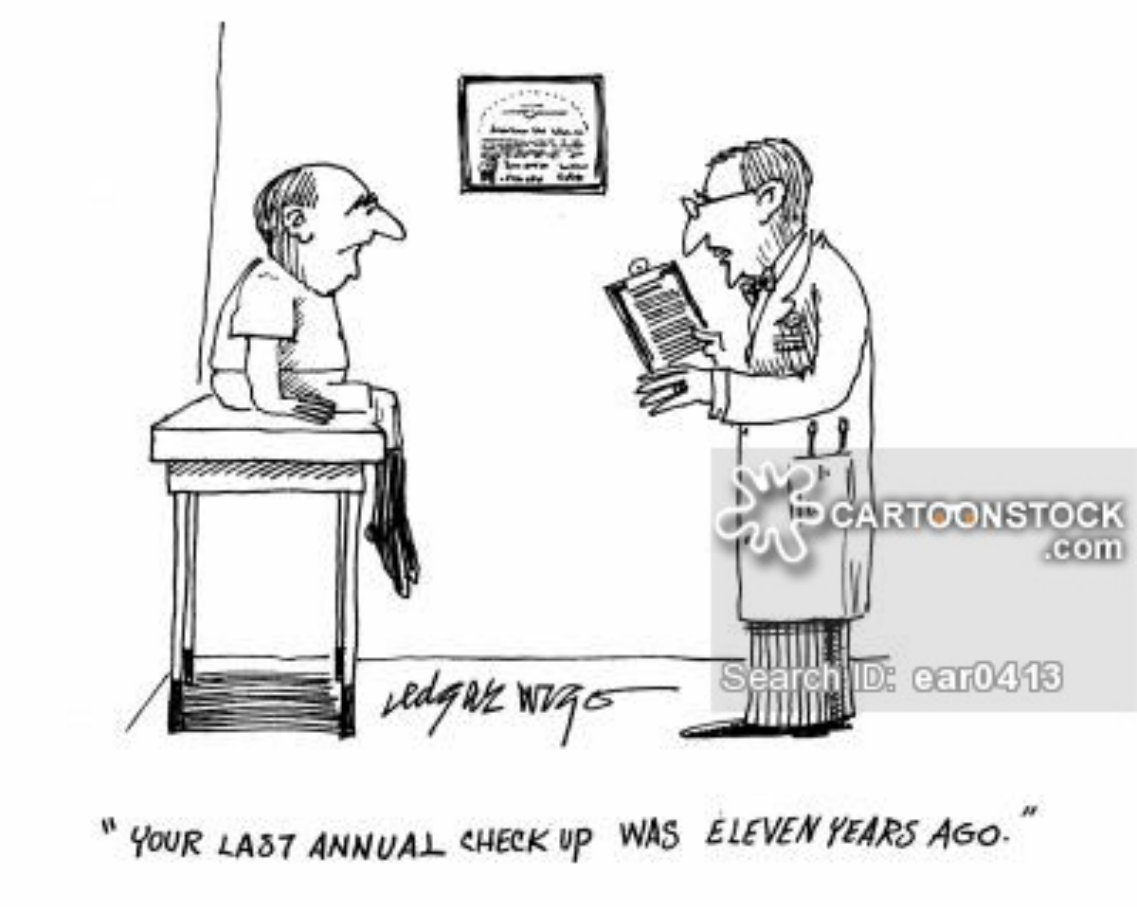
- **Initial Comprehensive Assessment**
- **Ongoing or partial assessment**
- **Focused or Problem-Oriented Assessment**
- **Emergency assessment**

- Initial Comprehensive Assessment
  - When:
    - Performed within specified time after admission to a health care agency
  - Why:
    - To establish a complete database for problem identification, reference, and future comparison

"Intensive and/or expensive care?"

Types of Assessment

- On going/Partial Assessment
  - consists of data collection that occurs after the comprehensive database is established.
  - This consists of a mini-overview of the client's body systems and holistic health patterns as a follow-up on health status.
  - Any problems that were initially detected in the client's body system or holistic health patterns are reassessed to determine any changes (deterioration or improvement) from the baseline data



Types of Assessment

- Problem-focused Assessment
  - performed when a comprehensive database exists for a client who comes to the health care agency with a specific health concern
  - A focused assessment consists of a thorough assessment of a particular client problem and does not cover areas not related to the problem.

Types of Assessment

- Emergency Assessment
  - When:
    - During any physiological or psychological crisis of the client
  - Why:
    - To identify life-threatening problems and to identify new or overlooked problems

Steps in Health Assessment

- Data Collection
- Validation of data
- Documentation of data

Data Sources

Primary Data Source

- Patient
  - Subjective Data
  - Objective Data

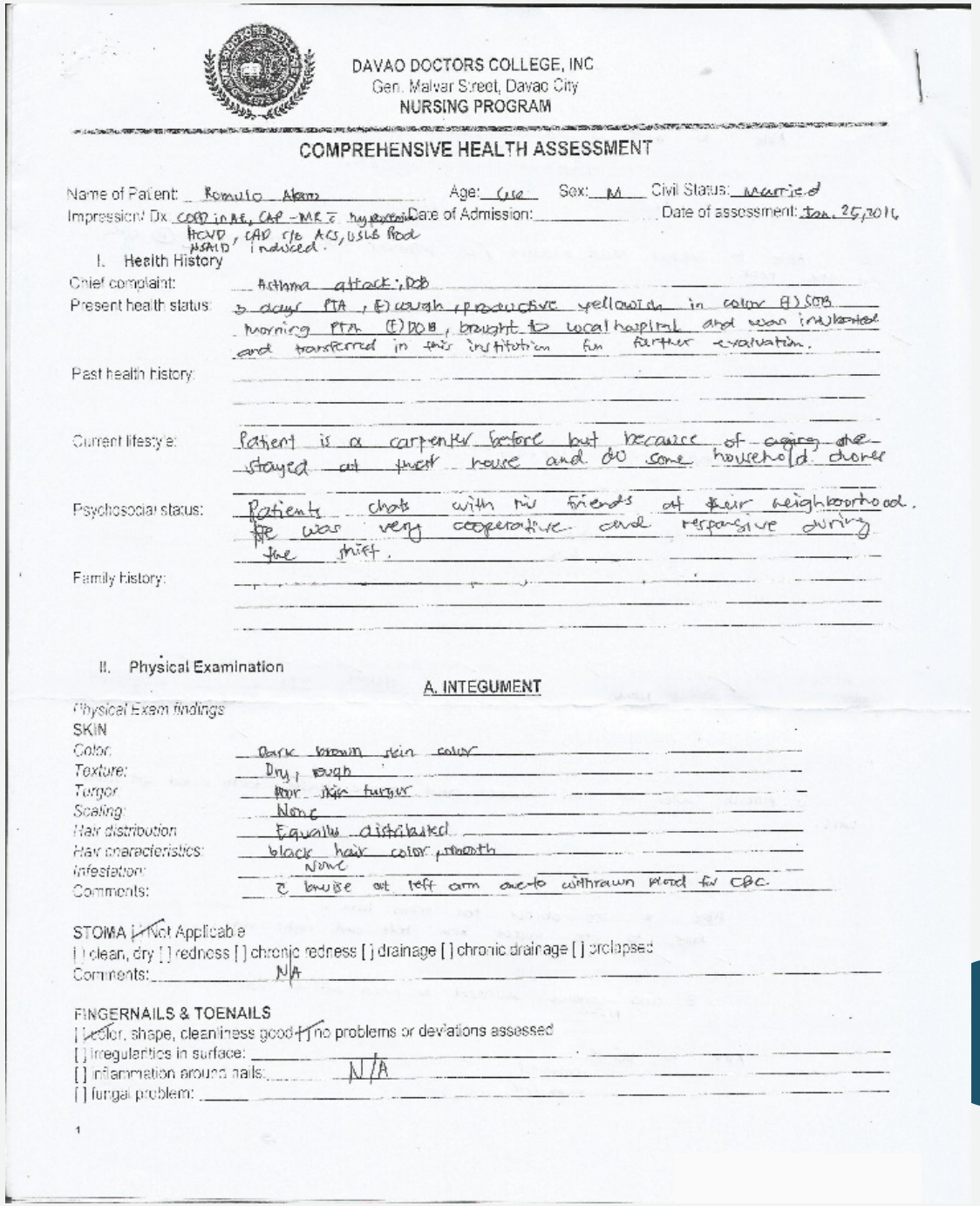
Secondary Data Source

- Family and Significant Other
- Health Care Team
- Medical Records
- Other Records and Related Literature
- Nurse's Experience

Data Collection Methods

Organizing the Data

- uses a written (or electronic) format that organizes the assessment data systematically
- often referred to as:
  - a nursing health history
  - nursing assessment
  - nursing database form



DAVAO DOCTORS COLLEGE, INC
Gen. Malvar Street, Davao City
NURSING PROGRAM

COMPREHENSIVE HEALTH ASSESSMENT

Name of Patient: Romulo Alam Age: 61 Sex: M Civil Status: Married
Impression/ Dx: COPD in AE, CAP - MR & hypertension, HCVD, CAD s/p ACS, USLB fed induced. Date of Admission: Date of assessment: Jan. 25, 2016

I. Health History

Chief complaint: Asthma attack; DOB

Present health status: 3 days PTA, (+) cough productive yellowish in color (+) SOB, morning PTA (+) DOB, brought to local hospital and was intubated and transferred in this institution for further evaluation.

Past health history:

Current lifestyle: Patient is a carpenter before but because of aging, he stayed at just house and do some household chores

Psychosocial status: Patients chats with his friends at their neighborhood. He was very cooperative and responsive during the shift.

Family history:

II. Physical Examination

A. INTEGUMENT

Physical Exam findings
SKIN
Color: dark brown skin color
Texture: Dry, rough
Turgor: Poor skin turgor
Scaling: None
Hair distribution: Equally distributed
Hair characteristics: black hair color, smooth
Infestation: None
Comments: 2 bruise at left arm due to withdrawn blood for CBC

STOMA [✓] Not Applicable
[ ] clean, dry [ ] redness [ ] chronic redness [ ] drainage [ ] chronic drainage [ ] prolapsed
Comments: N/A

FINGERNAILS & TOENAILS
[✓] color, shape, cleanliness good [✓] no problems or deviations assessed
[ ] irregularities in surface:
[ ] inflammation around nails: N/A
[ ] fungal problem:

Data Validation

- Validation is the act of "double-checking" or verifying data to confirm that it is accurate and factual

- Ensure that assessment information is complete.
- Ensure that objective and related subjective data agree.
- Obtain additional information that may have been overlooked.
- Differentiate between cues and inferences.
- Avoid jumping to conclusions and focusing in the wrong direction to identify problems.

Prioritizing Data

- consider the acuity of the problem, the patient's perception of the problem, and the situation at hand.
- Put life-threatening problems that require immediate attention at the top of the list, then problems affecting basic needs that require prompt attention, and finally, problems affecting psychosocial needs

Prioritizing Data

- Top-priority or **primary problems**—such as airway problems—are life-threatening.
- **Secondary problems**—such as pain—require prompt attention to prevent further progression or deterioration in your patient's condition.
- Although important, **third-level problems**—such as teaching needs—do not require immediate attention and can be addressed once your patient's condition has stabilized.
- Although actual problems usually take precedence over potential ones, potentially serious problems may also have high priority. Also consider the nature and context of the problem.

Consideration in HA

- **Special Considerations:**
  - **Age of the Client**
  - **Cultural Background of the Client**
  - **Emotion of the Client**
  - **Role of the Family**
- **Ethico-Legal Considerations**
  - **Informed Consent**
  - **Patients' Bill of Rights**
  - **Data Privacy Act**

The Nursing Interview

1. Pre-Introductory Phase
2. Introductory Phase
3. Working Phase
4. Summary and Closing Phase

- ***Pre-introductory Phase***
  - Review of client's medical record prior to initial contact if available
- ***Introductory Phase***
  - Introduction of self to the client
  - Citing the purpose of the interview
  - Discussion of the types of questions that may be raised
  - Explaining the reason of taking notes
  - Assurance of confidentiality
  - Ensure privacy

The Nursing Interview

1. Pre-Introductory Phase
2. Introductory Phase
3. Working Phase
4. Summary and Closing Phase

- ***Working Phase***
  - Gathers data (biographic data, chief complaints, health history, etc.)
  - Listens to client and observes for cues
  - Collaboration of both parties
- ***Summary and Closing Phase***
  - Summarizes set of information
  - Identification of possible plans
  - Clarification is encourage should there be any question in the gathered data or the identified possible plans

Communication

- Any means of exchanging information or feelings between two or more people. It is a basic component of human relationships, including nursing.
- Can be **verbal** and **non-verbal**

Verbal

- Use of written or spoken words to express oneself

Non-verbal

- sometimes called ***body language***
- Includes gestures, body movements, touch, physical appearance, facial expression, attitude, silence posture and gait.

Type of Questioning

- **Open-ended question**
  - How may/can I help you?
  - Where have you looked already?
  - What kind of information about your care are you looking for?

Type of Questioning

- **Closed-ended questions**
  - **Closed ended questions** are dichotomous **questions** often leading to yes or no answer.
  - **Closed-ended questions** generally include probing, or leading **questions**.
  - These **questions** are can mostly answered in a few words.

Type of Questioning

- **Laundry List**
- This **laundry list** approach helps you to obtain specific answers and reduces the likelihood of the client's perceiving or providing an expected answer.
  For **example**, “Is the pain severe, dull, sharp, mild, cutting, or piercing?” “Does the pain occur once every year, day, month, or hour?” Repeat choices as necessary.

Type of Questioning

- **Rephrasing**
- This technique helps you to clarify information the client has stated; it also enables you and the client to reflect on what was said. For example, your client, Mr. G., tells you that he has been really tired and nauseated for 2 months and that he is scared because he fears that he has some horrible disease. You might rephrase the information by saying, “You are thinking that you have a serious illness?”

Type of Questioning

- **Well-placed phrases**
- The nurse can encourage client verbalization by using wellplaced phrases. For example, if the client is in the middle of explaining a symptom or feeling and believes that you are not paying attention, you may fail to get all the necessary information. Listen closely to the client during his or her description and use phrases such as “um-hum,” “yes,” or “I agree” to encourage the client to continue.

Type of Questioning

- **Inferring**
- Inferring information from what the client tells you and what you observe in the client's behavior may elicit more data or verifying data. An example of inferring information follows: Your client, Mrs. J., tells you that she has bad pain. You ask where the pain is, and she says, "My stomach." You notice the client has a hand on the right side of her lower abdomen and seems to favor her entire right side. You say, "It seems you have more difficulty with the right side of your stomach" (use the word "stomach" because that is the term the client used to describe the abdomen). This technique, if used properly, helps to elicit the most accurate data posssible for the client.

- **Providing Information**

Type of Questioning

- **Providing Information**
- Providing Information Another important thing to do throughout the interview is to provide the client with information as questions and concerns arise. Make sure that you answer every question as thoroughly as you can. If you do not know the answer, explain that you will find out for the client. The more clients know about their own health, the more likely they are to become equal participants in caring for their health. As with nonverbal communication, several verbal techniques may hinder effective communication and should be avoided.

History Taking

Purpose:

- Directs focus of the physical exam
- Lays the groundwork for identifying nursing problems
- Provide and information that assists the examiner in identifying client's areas of strength and limitations

- **Biographic Data**
- **Chief Complaint**
- **History of Present Illness**
- **Past (Personal) History**
- **Gynecologic and Obstetric History**
- **Family History**
- **Review of System for Current Health Problems**
- **Lifestyle and Health Practices Profile**

Biographic Data

- includes information that identifies the client (name, age, address, sex, etc.)
- When students are collecting the information and sharing it with instructors, addresses and phone numbers should be deleted and initials used to protect the client’s privacy.

Chief Complaint

- reason/s for seeking care, this is what prompted the client for a check-up/admission

History of Present Illness

- Explore the chief complaint in more detail
- May follow PQRST, COLDSPA, LIQUORAAA or other methods

History of Present Illness

- **P -** Precipitating or Palliative Factors
- **Q -** Quality or Quantity
- **R -** Region / Radiation or Related Symptom
- **S -** Severity
- **T -** Timing

History of Present Illness

- **C – Character**
- **O – Onset**
- **L – Location**
- **D – Duration**
- **S – Severity**
- **P – Pattern**
- **A – Associated Factors**

Past (Personal) History

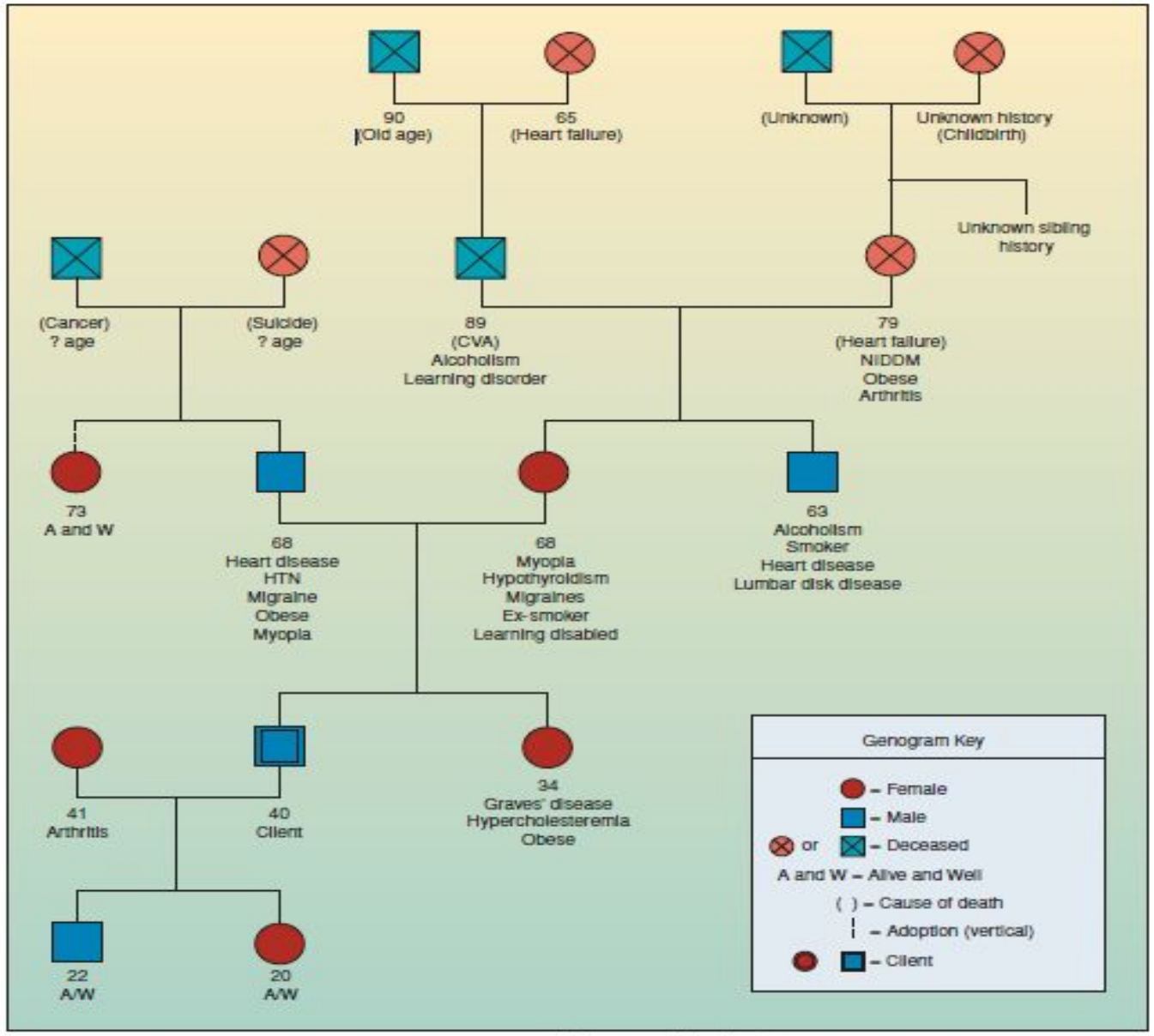
- Childhood illnesses, immunization status
- Adult illnesses (physical, emotional, mental)
- Past hospitalizations, surgeries and or accidents
- Chronic pain characteristics
- Allergies
- Medications (prescribed and OTC)

Gynecologic and Obstetric History

- Menarche, regularity of menstrual cycle, LMP, use of contraceptives
- History of previous and present pregnancy (GPA, TPAL, EDC)
- 10-27-22
- -3+7+1
- 7-34+23
- 8=3=23

Family History

- Health problems that seem to run in families and that are genetically based
- Presented through a genogram and narrative


90
(Old age)
65
(Heart failure)
(Unknown)
Unknown history
(Childbirth)
Unknown sibling
history
(Cancer)
? age
(Suicide)
? age
89
(CVA)
Alcoholism
Learning disorder
79
(Heart failure)
NIDDM
Obese
Arthritis
73
A and W
68
Heart disease
HTN
Migraine
Obese
Myopia
68
Myopia
Hypothyroidism
Migraines
Ex-smoker
Learning disabled
63
Alcoholism
Smoker
Heart disease
Lumbar disk disease
41
Arthritis
40
Client
34
Graves' disease
Hypercholesteremia
Obese
22
A/W
20
A/W
Genogram Key
– Female
– Male
or – Deceased
A and W – Alive and Well
( ) – Cause of death
– Adoption (vertical)
– Client

Review of System for Current Problems

- a brief review of essential functioning of various body parts or systems
- Each body system is addressed and the client is asked specific questions to draw out current health problems or problems from the recent past that may still affect the client or that are recurring.
- Care must be taken in this section to include only the client's subjective information and not the examiner's observations.

Lifestyle and Health Practices Profile

- **Activity Level and Exercise**
- **Description of a typical Day**
- **Sleep and Rest**
- **Nutrition and Weight Management**
- **Substance Use**
- **Self-Concept and Self-care Responsibilities**
- **Social Activities**
- **Relationships**
- **Values and Belief System**
- **Education and Work**
- **Environment**
- **Stress Levels and Coping Styles**

Conducting Physical Examination (PE)

1. Preparation of the setting, oneself and client for PE
2. Positioning the client appropriately
3. Types of and operation of equipment needed for a particular examination
4. Performance of the 4 assessment techniques

Preparation

- **Ensure to meet the following:**
  - Comfortable and warm room temperature
  - Private area, free from interruptions and distractions
  - Adequate lighting
  - Firm examination table
  - Bedside tray for equipment
  - Collect all necessary equipment prior to PE
  - Prevent transmission of infectious agents by following standard precaution

Preparing the Client

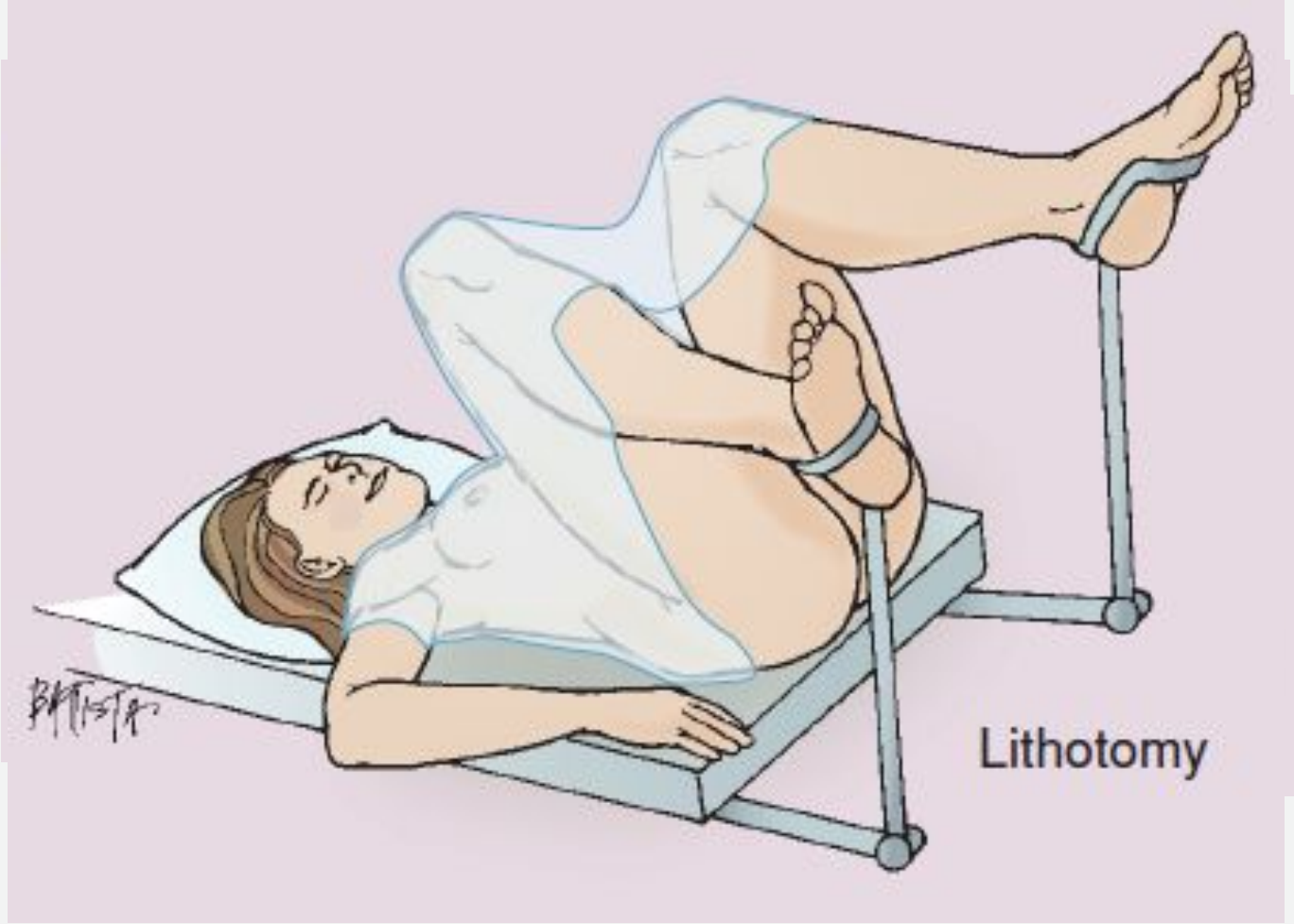
- Nurse-client relationship should be established during client interview before the PE
- Describe to the client what the examination will involve
- Respect client's desires and requests related to the PE
- Explain to the client the purpose of collecting specimen (if necessary)

Preparing the Client

- Ensure client's privacy
- Begin examination with less intrusive procedures such as taking the vital signs
- Approach client from the right side since most techniques are performed with the examiner's right hand
- Depending on the procedure, the client may be asked to change positions frequently

Positioning the Client

- Standing
- Sitting
- Supine
- Prone
- Dorsal Recumbent
- Sim's
- Knee-chest
- Lithotomy



Physical Examination (PE) Techniques

1. **Inspection**
2. **Palpation**
3. **Percussion**
4. **Auscultation**

Inspection

- Using senses of vision, smell, and hearing to observe and detect any normal or abnormal findings.
- May require the use of a special equipment (e.g. ophthalmoscope)
- Make sure the room is a comfortable temperature. Use good lighting
- Look and observe before touching
- Completely expose the body part you are inspecting while draping the rest
- Note color, patterns, size, location, consistency, symmetry, movement, behavior, odors or sounds

Palpation

- Using parts of the hand to touch and feel for texture, temperature, moisture, mobility, consistency, strength of pulses, shape, and degree of tenderness
- Parts of the Hand used when palpating:
  - **Fingerpads**: Fine discriminations, pulses, texture, size, consistency, shape, crepitus
  - **Ulnar/Palmar surface**: vibrations, thrill, fremitus
  - **Dorsal Surface**: temperature

Palpation

- **4 types of palpation**:
  - ***Light*:** very little or no depression (less than 1cm)
  - ***Moderate*:** depress skin 1-2 cm, use circular motion
  - ***Deep*:** surface depression between 2.5 – 5 cm, for deep organs
  - ***Bimanual*:** using 2 hands on each side of body part; the other hand for applying pressure and the other for feeling the structures

Percussion

- Involves tapping body parts to produce sound waves.
- These sound waves or vibrations enable the examiner to assess underlying structures.
- Types of Percussion
  - ***Direct***: direct tapping of a body part, elicits possible tenderness
  - ***Blunt*:** To detect tenderness over organs (e.g. kidneys) by placing one hand flat on the body surface and using the fist of the other hand to strike the back of the hand flat on the body surface
  - ***Indirect*:** Elicits sounds that give clues to the makeup of the underlying tissue.

TABLE 3-3 Sounds (Tones) Elicited by Percussion

Sound	Intensity	Pitch	Length	Quality	Example of Origin
Resonance (heard over part air and part solid)	Loud	Low	Long	Hollow	Normal lung
Hyper-resonance (heard over mostly air)	Very loud	Low	Long	Booming	Lung with emphysema
Tympany (heard over air)	Loud	High	Moderate	Drum-like	Puffed-out cheek, gastric bubble
Dullness (heard over more solid tissue)	Medium	Medium	Moderate	Thud-like	Diaphragm, pleural effusion, liver
Flatness (heard over very dense tissue)	Soft	High	Short	Flat	Muscle, bone, sternum, thigh

Auscultation

- Requires the use of a stethoscope to listen for heart sounds, movement of blood through the cardiovascular system, movement of the bowel, and movement of air through the respiratory tract.
- Eliminate distracting noises from the environment
- Expose part that is to be auscultated. Do not auscultate over clothing.

General Survey

- The first step in head-to-toe assessment
- Involves observation of the client's general appearance, level of comfort, mental status, measurement of vital signs, height and weight
- Often done simultaneously while taking patient's health history

Mental Status Exam

- Represents a cross section of a person's psychological life sum total of the nurse's assessment at the moment
- Purpose: to construct a picture of client's cognitive, affective and psychomotor capacity, status and condition

General Survey

- General Appearance
- General Mobility
- Facial Expression
- Activity
- Behavior
- Thought Process and Thought Control
- Emotional State and Reaction
- General Sensorium and Intellectual Status

Vital Signs: Normal Values

- **Temperature**
  - Usual Normal Range: **36°C to 37.5°C**
  - Elevated: **Hyperthermia (>37.5°C)**
  - Below: **Hypothermia (<36°C)**
  - Routes and normal ranges
    - Oral: 35.9°C – 37.5 °C
    - Rectal: 36.3 °C – 37.9 °C
    - Axillary: 35.4°C – 37°C
    - Tympanic: 36.7 °C – 38.3 °C
    - Temporal: 36.3 °C – 37.9°C

Vital Signs: Normal Values

- **Pulse Rate**
  - Adult Normal Range: **60-100 bpm**
  - Elevated: **Tachycardia (>100 bpm)**
  - Below: **Bradycardia (<60 bpm)**
  - Normal ranges (age group)
    - Infant: 120 – 160 bpm
    - Toddler: 90 – 140 bpm
    - Preschooler: 80 – 110 bpm
    - School age: 75- 100 bpm
    - Adolescent: 60 – 90 bpm
    - Adult: 60 – 100 bpm

Vital Signs: Normal Values

- **Respiratory Rate**
  - Adult Normal Range: **12-20 cpm**
  - Elevated: **Tachypnea (>20 cpm)**
  - Below: **Bradypnea (<12 cpm)**
  - Normal ranges (age group)
    - Newborn: 30 – 60 cpm
    - Infant: 30 – 50 cpm
    - Toddler: 25 – 32 cpm
    - Child: 20 – 30 cpm
    - Adolescent: 16 – 20 cpm
    - Adult: 12 – 20 cpm

Vital Signs: Normal Values

- **Blood Pressure**
  - Adult Normal Range: **90/60 – 120/80mmHg**
  - Elevated: **Hypertension (>120/80 mmHg)**
  - Below: **Hypotension (<90/60 mmHg)**
  - Classification:
    - Hypotension: <90/60 mmHg
    - Normal: 90/60 – 120/80 mmHg
    - Prehypertension: 120-139/80-89 mmHg
    - Stage 1 Hypertension: ≥ 140/90 mmHg
    - Stage 2 Hypertension: ≤ 169/90 mmHg

Vital Signs: Normal Values

- **Respiratory Rate**
  - Adult Normal Range: **12-20 cpm/ bpm**
  - Elevated: **hyperpnea (>20 cpm)**
  - Below: **Bradypnea (<12 cpm)**
  - Normal ranges (age group)
    - Newborn: 30 – 60 cpm
    - Infant: 30 – 50 cpm
    - Toddler: 25 – 32 cpm
    - Child: 20 – 30 cpm
    - Adolescent: 16 – 20 cpm
    - Adult: 12 – 20 cpm